

Breech baby

At 32 weeks, 15 percent of babies are breech (bottom first). With time, most turn, with just 3 percent remaining breech at term.

Breech labors and births are usually more difficult, so if your baby is still in a breech position in late pregnancy, you may be offered a procedure to turn your baby.

Turning your baby

The procedure to turn your baby, cephalic version (ECV), is usually offered at around 37 weeks. Using ultrasound for guidance, the doctor presses on the lower part of your belly to elevate your baby's bottom out of the pelvis. Pressure is used to rotate your baby until they head down. The procedure may feel uncomfortable, and you may be given medication to relax the uterus. A scan is often done before and after the procedure.

The success rate for ECV is 40–60 percent and complications are rare. If an ECV is unsuccessful and you want to try for a vaginal birth, your doctor will discuss the risks of, which are higher than if a Cesarean delivery is planned.

Breech diagnosed in labor

It can be hard to distinguish between a baby's head and bottom by feeling the tummy, so occasionally a breech baby isn't discovered until labor. If a breech is diagnosed in labor, you can still have an ECV, but at this stage it has a lower success rate. An ECV may not be possible in labor if the water has broken, in advanced labour, or if there's no one to do the procedure.

Delivery of your baby

If you try for a vaginal birth, you should be in a hospital with an ob-gyn present, since you may need an assisted delivery or an emergency Cesarean. Fetal monitoring should be done throughout labor, since babies in distress will need to be delivered by Cesarean. Sometimes a breech baby has problems getting oxygen and blood through the umbilical cord, if it's squeezed into the wrong position.

You'll be advised to have your baby's heartbeat monitored throughout, and you may have an IV in case you need a Cesarean. You may be asked to lie on your back with your feet in stirrups so that the doctor can access the baby. An alternative position is to stand or be on all fours. Your bladder may be emptied with a catheter and an [episiotomy](#) cut may be done to prevent a more serious tear. The doctor may apply gentle pressure to the baby's arms or legs during birth, but you're unlikely to notice this. Except for these differences, the birth is likely to feel the same as the birth of a head-first baby. During the birth, the head will be eased out of the birth canal either by hand or with forceps to control the speed of the birth. An emergency Cesarean is likely if any complications arise.

POSITIONS OF A BREECH BABY

A breech baby can lie in one of three positions, known as extended (frank); flexed (complete); or footling. With an extended breech, the hips are flexed, knees extended, and the feet are by the head. A vaginal delivery is most likely with this type of breech. With a flexed breech, the hips and knees are flexed, with the feet above the buttocks, and a vaginal birth may be possible. In a